



Clinical Care of People Who May Be Suicidal Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RDBG)	All inpatient, outpatient, and community clinical areas	Medical, Nursing, Allied Health, Peer Workers

Policy statement

The purpose of this policy is to guide clinical decision-making across all clinical areas to support the safety and recovery of people who may be suicidal.

Appropriate clinical care and management is to be integrated and coordinated across all relevant RDBG Divisions and provided in line with the care outlined in this document and the following WA Department of Health documents:

- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#)

Key points

- Not all people who experience suicidal thoughts and behaviours have a mental illness and not all people who experience mental illness attempt suicide.
- A person's suicidality or self-harm behaviour can fluctuate in duration and intensity, over small periods of time, and can have significant impact on changes in mental state and risk.
- Populations in high risk groups include people with any history of previous suicidal behaviour, Aboriginal people and sexual, gender and bodily diverse people. Referral to or liaising with clinicians from specialist areas is required where possible.
- Extensive, global evidence of an increased risk for suicide attempts and completed suicide following discharge from inpatient care. ^{1, 2}

For the purpose of this document

- Personal support persons (PSPs) include family, carer, friend, significant other, guardianship and next of kin [NoK]
- Patients, clients, and consumers are terms used interchangeably and will be referred to as patients in this policy.

Contents

Initial risk assessment.....	3
General health clinicians.....	3
Mental health treating clinician or treating team	4
Patient safety management	4
General health and mental health clinicians	4
Mental health assessment and safety planning	5
Ongoing treatment and safety planning	6
General health and MH clinicians	6
MH treating clinician or treating team – additionally.....	6
Patients with high risk or recurrent presentations of suicidal thoughts and/or behaviours ...	7
General Health and MH clinicians.....	7
MH treating clinician or treating team – additionally.....	7
Management following suicidal incidents or a suspected death by suicide.....	8
General Health and MH clinicians.....	8
Staff and patient support.....	8
Transfer or discharge planning	9
General Health and MH clinicians.....	9
MH treating clinician or treating team - additionally	10
Day of discharge	10
General Health clinicians	10
MH treating clinician or treating team	10
Post-discharge follow up.....	11
CMHS treating clinician or treating team	11
Compliance monitoring	11
References	14
Appendix I: Examples of clinical interviewing questions for persons who may be suicidal	15

Policy directives

Care provision is:

- Compassionate, trauma-informed, recovery oriented, person centred, culturally responsive, respectful, nonjudgmental, and developmentally appropriate
- Collaborative between staff, patients, PSPs where possible and with consent, and across services where required
 - e.g. MH and ED clinicians are to work in partnership – refer to **Inpatient Admissions from the ED Policy**

Admissions from the ED Policy

- Delivered in a safe physical environment when on RPBG sites
- Comprehensively documented in patient progress notes (DMR) and service specific records e.g. Multidisciplinary Comprehensive Care Plan and What Matters Most Record, SSCD, PSOLIS as appropriate

General health clinicians are:

- Able to recognise and effectively respond to behaviours of concern
 - Including timely referral to specialist services to support early access to mental health assessment, care planning and treatment

Mental health (MH) clinicians are:

- Able to recognise and effectively respond to mitigate risk relating to patients who may be suicidal
 - Including completion of suicide prevention formulation as part of safety planning

Training:

- To be provided to clinicians responding to patients who may be suicidal to support suicide safety assessment and management in alignment with the [WA Department of Health Safety Planning Procedures for Mental Health Consumers](#) and [EMHS Mandatory Training Quick Reference Guide](#)
 - MH clinicians to be able to assess, advise and implement effective strategies aiming to prevent suicide and facilitate access to appropriate care.
 - Recommend training, where applicable, of:
 - Champions – promoters and implementers of specific initiatives to improve patient care, and
 - Mentors – socialisers, role models and educators.

Guidelines

Initial risk assessment

General health clinicians

- Escalate risk or concern to senior clinician and medical staff (where relevant) for referral to specialist services for MH assessment
 - Refer to [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition and Behaviour SOP](#) – Appendix I: Specialist Referral Pathways – General Health.

Guidelines – Initial risk assessment cont'd

Mental health treating clinician or treating team

- Assess and document risk as soon as possible using relevant assessment tool(s) (e.g. *Risk Assessment and Management Plan [RAMP]*)
- Ask clear direct questions to assess risk, ideation, intent, hopelessness, and potential triggers
 - Refer to [Appendix I: Examples of clinical interviewing questions for persons with suicidal risk.](#)
- Escalate risk to senior clinician and medical staff (where relevant) according to [RRAD in Mental State, Cognition and Behaviour SOP](#) Appendices II – VI.

Patient safety management

General health and mental health clinicians

- Apply distraction and de-escalation techniques
- Provide access to an appropriate location within the clinical area where possible (e.g. least busy area, away from obvious ligature points)
- Restrict or prohibit access to objects which could be used for self-harm and/or suicide where relevant (e.g. ask if patient has any medication on their person, facilitate removal of belt, shoelaces in inpatient areas)
- Offer supportive resources and encourage self-help management where possible
- Consider administration of appropriate medication where relevant - e.g. inpatient areas
- Ensure patient is adequately supervised and observed for the level of risk identified e.g. harm to self or leaving a clinical area without notice
 - Keep patient within line of sight as a minimum and consider 1:1 Special or other levels as required as per **Special and Supportive Observations Policy**
 - Flag patient as a patient of concern on i.CM
- If the patient initiates discharge, leaves without notice or discharges against medical advice (DAMA) following, during, or prior to assessment, escalate to senior clinician and actively respond as per relevant EMHS policy, **e.g. Acute Response Mental Health Emergencies/Crisis Policy**
- Escalate to senior clinician for careful consideration regarding any already planned and authorised episodes of leave where relevant
- Facilitate a collaborative approach with multidisciplinary team (MDT), across general health and MH services and with community MH services (CMHS), general practitioners (GPs), and/or other services as required, e.g. Aboriginal Health Liaison Officers (AHLOs), LGBTIQ+ allies, cultural supports
- Where medical or MH review has not occurred within agreed time frame, escalate to senior staff (refer to **EMHS Acute Response Mental Health Emergencies/Crisis Policy**).

Guidelines cont'd

Mental health assessment and safety planning

- Discuss with patient (and PSP where consent is provided), assessment and treatment processes and options
 - If a patient declines consent for PSP involvement in their care, staff are to periodically revisit this with a view to therapeutically challenge these preferences and document the outcome in the progress notes and WebPSOLIS, giving consideration to the patient's mental state, mood fluctuations and whether the patient is a voluntary or involuntary patient.

Note: Should circumstances arise where a clinician's duty of care to maintain a consumer's safety overrides the requirement to obtain consent to share information, immediately escalate to senior and medical staff

- Complete comprehensive, suicide-specific assessment to inform suicide prevention formulation and clinical judgement. General health areas to seek specialist input.
 - Utilise relevant tools to guide assessment
 - Consult with lead clinicians, PSPs, police etc. to corroborate history whenever possible
- Develop care plan in collaboration with MDT, consult psychiatric consultant liaison or on call mental health team as required and risk mitigation plan with patient as soon as is possible, e.g. use a *What Matters Most Record (WMMR)* to support self-management (inpatients), *Behavioural Observation Form* (general health areas) or *Crisis Awareness Plan* (Community MH) in the *Treatment Support Discharge Plan*, and a *Multidisciplinary Comprehensive Care Plan (MCCP)* or other care plans as applicable:
 - Registrar to liaise with the Consultant, particularly where the decision is made not to admit a patient to a MH inpatient unit where risks are unable to be mitigated
 - Clarify **Mental Health Act (MHA) 2014** responsibilities (e.g. the need for forms, notifications) with MO or Authorised MH practitioner. General health areas to seek specialist input
 - Consider identification of which suicide risk factors are modifiable and how they can be addressed - e.g. potential changes that could rapidly increase a patient's risk
 - Identify strategies to mitigate suicidal risk and enhance safety – e.g.
 - Interventions supporting adaptive coping skills
 - Removing or reducing potential access to lethal means, e.g. property checks and searches (refer to the following RPBG documents - **Potentially Harmful Items Management for Mental Health Patients SOP** and **Patient Property Management Policy**)
 - Escalate to senior clinician any requested episodes of leave
 - Consider potential for sexual safety concerns (refer to **RPBG Sexual Safety Policy**)
 - Identify and agree how the patient's safety will be monitored by the patient, staff, and PSP (e.g. appropriate observation)
 - Document clear actions, roles and responsibilities for staff and an escalation plan to action in the event of a crisis or acute deterioration

Guidelines – Mental health assessment and safety planning cont'd

- Document where agreement with patient and PSP cannot be reached
- Provide education, information and/or referral, as necessary, to services within and outside of RPBG health e.g. online self-help resources, emergency contacts
- Commence discharge and follow-up planning as early as possible
- Ensure timely review of safety plans and additionally update as required e.g. following a crisis or suicide attempt, changes in presentation and context or availability of support
- Document all of the above in the patient's integrated notes and relevant service patient records (e.g. SSCD, PSOLIS) according to local processes.

Ongoing treatment and safety planning

General health and MH clinicians

- Continue to prioritise safety and wellbeing of the patient at risk of suicide and staff
- Routinely seek professional consultation
- Request medical review as applicable.
- Complete review of care and safety plans following an incident or during periods of heightened risk, otherwise indicated
- Consider referral to MH Division Allied Health services where relevant (e.g. inpatient social work or clinical psychologists) or RPBG community Clinical psychology services, external, counselling or supports. Refer to **eReferral Process: Inpatient and Outpatient SOP**

Note: There is strong evidence supporting the use of suicide-specific psychological treatments, e.g. Dialectical Behaviour and Cognitive Behavioural therapies for suicide risk³ in collaboration with other therapy and treatment regimens¹.

- Document comprehensively all treatment in the relevant care plan.

MH treating clinician or treating team – additionally

- Manage acute suicidal crises
 - Identify brief interventions able to be actioned immediately to support a patient's safety and ability to manage own suicide risk
 - Ensure medical management is accessed where required - including of near-lethal hangings
 - Promptly addressing pain and withdrawal symptoms is critical
- Continue to regularly review and update suicide-specific treatment care plan ensuring direct targeting and treatment of problems and issues compelling the person to consider suicide in consultation with the patient, PSP, and wider supports (e.g. GP). Refer to [Community Mental Health Services \(CMHS\) Triage to Discharge SOP](#) Appendix I Traffic Light System.
- Complete medical reviews of the patient as per the **Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Mental Health (MH) Wards or CMHS Policy**
- Documentation also to be consistent with the MHA 2014 and Statewide Standardised Clinical Documentation (SSCD).

Guidelines cont'd

Patients with high risk or recurrent presentations of suicidal thoughts and/or behaviours

General Health and MH clinicians

- Assess risk at each presentation and review throughout episode of care
- Escalate to senior clinician or medical staff and as per the [RRAD in Mental State, Cognition and Behaviour SOP](#) and/or [RRAD Policy](#)
- Communicate between staff, across relevant services and with patient and PSP where possible and with consent
 - e.g. MH and ED – refer to **Inpatient Admissions from the ED Policy**
- Actively respond to all co-existing conditions
- Facilitate removal or reduction of environmental hazards in inpatient facilities (particularly MH services)
- Local open disclosure processes to be consistent with the [Clinical Incident Management and Open Disclosure SOP](#), including the provision of ongoing support for PSPs and clinicians

MH treating clinician or treating team – additionally

- Requirement to develop clear strategies to support the patient's recovery, respond to changes in risk over time and contain emotional distress
 - Exploring the patient's coping strategies – e.g.
 - Consider the historical and dynamic nature of risk
 - Assess patient capacity (refer to [Assessing Patient Capacity SOP](#))
 - Review patient's support network
 - Integrating clinical and non-clinical care and supports (including case conferencing)
 - Regularly reviewing care and safety plan(s)
 - Applying a risk-averse management lens
 - Ensuring MDT approach to risk formulation and response
 - Acknowledging the underlying distress that drives suicidal ideation
 - Setting clear expectations of suicide-specific assessment and a clear management plan, including expected behaviour of the patient and/or PSPs
 - Facilitate the patient's engagement with and linkage to programs that promote emotional and problem-solving skills.

Guidelines cont'd

Management following suicidal incidents or a suspected death by suicide

General Health and MH clinicians

- In an emergency situation where a patient has attempted to self-ligate, a ligature cutter may need to be used rapidly for the preservation of life or limb. Refer to the **Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy**
- Complete a Datix Clinical Incident Management System (CIMS) and/or Combined Hazard or Injury Report (CHoIR) for self-harm and suicidal clinical incidents within 24 hours of a known incident
- Senior Nurse on Call and Chief Psychiatrist to be notified of attempted suicides or all deaths of MH patients due to suspected suicide or overdose via the *Notifiable Incident Reporting Form* within 24 hours (see [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist – Policy for Public Mental Health Services 2018](#))
 - Demonstrate compassion, openness, respect, and empathy to the PSPs of a person who has died
 - Communicate with PSPs after an incident in care as per the [Clinical Incident Management and Open Disclosure SOP](#)
 - Advise any clinician (including private psychiatrists and GPs) or nonclinical psychosocial support service who has been managing care of the deceased in the community of the death as soon as possible
- In circumstances of a person's death
 - Medical staff /Consultants to complete relevant Morbidity and Mortality documentation – refer to **Morbidity and Mortality Reviews SOP**
 - Available senior management (e.g. Head of Department [HoD], Nurse Unit Manager (NUM), On Call Psychiatrist, After Hours BHS MH Duty CNS) to ensure a clinical investigation occurs at the time of the person's death
 - Refer to *RPMG Death in Hospital Form* and **End of Life CPS** for details relating to reporting the death to the Coroner and that the person's body must remain as it is at the time of death until permission to provide appropriate care has been obtained.

Staff and patient support

- Patients or PSPs affected by an incident are to be followed up immediately and provided support as per [Mental Health Patient, Carer, and Visitor Support Post-Critical Incident SOP](#)
- Staff affected by an incident are to be followed up immediately by Line Managers and provided support as per [EMHS Post Incident Management/Critical Incident Stress Debriefing Policy](#).

Guidelines cont'd

Transfer or discharge planning

General Health and MH clinicians

- Follow requirements outlined in [EMHS Discharge](#) and [EMHS Discharge Communication](#) policies and the [WA Department of Health Safety Planning for Mental Health Consumers Policy](#)
- Conduct clinical handover to other service providers that combines written referrals with a person-to-person discussion using **iSoBAR**
- Incorporate follow-up into care and safety plans
- Record risk as an alert in PSOLIS (MH patients), iSOFT, WebPAS (services with access) and in patient integrated notes
- Ensure accurate and up to date contact details for the patient, PSPs and GP are documented in the patient's medical record
- Include collaborative communication with GPs, CMHS, other health service providers as relevant
- Apply careful consideration to any medication changes, ensuring the risks and side effects are discussed and adequate follow up is in place to monitor response and risks associated with the changes
- Complete discharge medications or prescriptions, wound care, equipment or medical certificates (as required) as per **Medication Supply to Non-Admitted or Discharged Patients by Prescribers SOP**
 - Consider supplying no more than one week of medication to minimise the risk of self-harm and to encourage follow up with the Community Team as appropriate
- Discharge will be preferably within person's catchment or local area to minimise potential risks e.g. interruption to care
- Arrange and confirm destination transport and confirm transport for person pick up, if required, and inform the person's PSP that the person is discharged and of their travel arrangements
- Discharge plan to address ongoing support services available for recurrent issues e.g. community or ED or helplines
- Where involved, Consultation-Liaison (C-L) Service to write in patient integrated notes regarding assessment, treatment, management, reviews, liaison undertaken with existing community services and facilitation of new referrals as appropriate – e.g. to community services, GP, Psychology
 - This information will inform the discharge summary
 - Discharge summary and plan to be sent to relevant CMHS
- Also refer to RPBG documents:
 - **General Health Admission, Transfer and Discharge SOP**
 - **Mental Health Inpatient Triage to Discharge SOP**
 - **CMHS Triage to Discharge SOP**
 - **Unplanned Transfer (BHS and TCU) SOP**
 - **Unplanned Presentations to BHS SOP**

Guidelines - Transfer or discharge planning cont'd

MH treating clinician or treating team - additionally

- Record risk as alert in PSOLIS
- The person's PSP must be involved in the discharge or transition planning unless the person's Consultant reasonably believes this is contra-indicated or the person refuses consent (Sec 188, MHA 2014)

Note: If there is clinical risk to the person, details of the discharge plan (i.e. key elements) may be provided to the person's PSP, without person's consent
If shared without consent, document in patient integrated notes

Day of discharge

General Health clinicians

- *Notifications and Clinical Summaries (NaCS)* to include:
 - Collaborative co-management summary reflecting relevant services involved (e.g. general health, C-L)
 - Actions for community services and/or GP as relevant
 - Referrals in progress
 - Medication information
 - Details and dates of follow up appointments
- Copy of NaCS to be given to patient, and PSP where relevant

MH treating clinician or treating team

- Provide a copy of a signed *Care Transfer Summary (CTS)* or *Discharge Summary* to the patient, PSP, and GP, CMHS or other services upon discharge that includes:
 - MH emergency and follow up information for MH inpatient discharges or ED MH patient presentations as per **EMHS Discharge Communication Policy** (see Additional requirements for MH inpatients and ED discharge sections) and [WA Department of Health Safety Planning for Mental Health Consumers Policy](#)
- In addition to the record of the above information in the *CTS* or *Discharge Summary*, upon discharge, a card is to be provided to the person with
 - The time and date of all appointments arranged or services the person has been referred to
 - Contact details of emergency services and out of hours contact numbers – including Mental Health Emergency Response Line (MHERL) and CAMHS for child or adolescent services.
- Encourage patient to photograph card in case card is lost.

Guidelines cont'd

Post-discharge follow up

CMHS treating clinician or treating team

- Make direct contact with MH persons discharged from an acute MH admission to the community within 7 days of discharge (mandatory time frame) – with a preference for phone contact within 48 hours and face to face contact within 7 days - or as agreed in the patient's safety and/or care plan. Refer to [WA Department of Health Safety Planning Procedures for Mental Health Consumers](#) and **EMHS Supporting transition from hospital [7 day follow up] Guidelines**)

Note: If there is indication of clinical deterioration and/or suicidal risk post discharge, more assertive prompt community follow up and assessment will occur ahead of scheduled appointments or potential admission.

- The person's GP where applicable, is to be involved in post discharge follow up. A discharge summary is to be sent to the GP within 24 hours
 - If person has no GP, prior to discharge staff are to support the person to engage with a regular GP and provide the person with information on available resources e.g. MHERL, Safe Haven, ED, other relevant community services; a referral to Assessment and Treatment Team can be made

Note: Referrals and CTSs are to highlight identified risk – particularly suicide-related risk, and urgency for required follow up

- If a MH patient does not attend a planned community appointment with an allocated clinician or Care Coordinator refer to the **Community Mental Health Services Did Not Attend (DNA) Community Appointment SOP**.

Compliance monitoring

- Self-harm, individual suicide and attempted suicidal clinical incidents reported in Datix CIMS and/or CHoIR and/or Office of the Chief Psychiatrist (OCP) notifiable incidents reports
- ED and MH services must
 - Conduct an annual review of all reportable suicide or attempted suicide incidents for each 12 month period as per [WA Department of Health Safety Planning Procedures for Mental Health Consumers](#)
 - The review report will be tabled at relevant governance committees to inform system improvement and identify and promote sharing of lessons learnt
- Reported clinical incidents will be reviewed monthly at relevant Divisional specific Clinical Quality and Safety meetings and by Coordinators of Nursing or Program Managers or HoDS.

Facilitators

Marika Spaseska, Policy Officer, Innovation and Education, RPBG

Authors / acknowledgements

We acknowledge the following previous site endorsed work and/or contributors used to compile this document: Armadale and Kalamunda Group Clinical Care of People Who May Be Suicidal Procedure, Fiona Stanley Fremantle Hospitals group Clinical Care of People Who May Be Suicidal, Alison Harrison, Coordinator Planning, Innovation and Commissioning – MH, EMHS

Review authors 2024

Kirsty Stewart, MHD, RPBG

Jason Ellis, Deputy Co-Director, Mental Health Division (MHD), RPBG

Kirstie Kemble, Safety and Quality Coordinator, MHD, RPBG

Alexandra McGlynn, Coordinator of Nursing, Emergency Department, RPBG

Judith Cuff, Safety and Quality Coordinator, HoLAA, RPBG

Teal Harris, A/Social Work Manager, Mental Health, BHS, RPBG

Hollie Van Schooten, A/Social Work Manager, BHS, RPBG

Brenda Jones, CNM, C-L Psychiatry/Addiction, RPBG

Lola Sikora, Trauma Program Manager, RPBG

Marika Spaseska, Policy Officer, Innovation and Education, RPBG

Andrew Wollan, CNS, CMHS, RPBG

Sarah MacKinnon, CoN Mental Health Division, RPBG

Feedback received 2024

Carli Beange, Midwifery Manager, Surgical Division, RPBG

Centre for Wellbeing and Sustainable Practice, RPBG

Sabrina Dowd, CNS, C-L Psychiatry/Addiction, RPBG

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Clinical Care of People Who May be Suicidal Policy through consultation with Aboriginal Health Strategy Director, Senior Project Officer Cultural Security, Aboriginal Health Strategy Team, Coordinator Aboriginal Health Liaison RPBG, Aboriginal Health Liaison Officer, and Specialist Aboriginal Mental Health Service Wungen Kartup to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 334).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policy, practice standard, clinical guidelines

RPMG Policy Hub

- [ED: Triage and Waiting Room SOP](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [General Health Admission, Discharge and Transfer SOP](#)
- [Prevention and Management of Aggressive Behaviours within MH Inpatient Settings SOP](#)
- [Behaviours of Concern \(Preventing, Recognising and Responding\) Practice Standard](#)
- [Unplanned Transfer \(BHS and TCU\) SOP](#)
- [Unplanned Presentations to BHS SOP](#)
- [Special and Supportive Observations Policy](#)
- [Inpatient Admissions from the ED Policy](#)
- [Potentially Harmful Items Management for MH Patients SOP](#)
- [Patient Property Management Policy](#)
- [Sexual Safety Policy](#)
- [Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Mental Health \(MH\) Wards or CMHS Policy](#)
- [eReferral Process: Inpatient and Outpatient SOP](#)
- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Morbidity and Mortality Reviews SOP](#)
- [Medication Supply to Non-Admitted or Discharged Patients by Prescribers SOP](#)
- [CMHS Did Not Attend \(DNA\) Community Appointment SOP](#)

EMHS Policy Hub

- [Care Coordination in Mental Health Services Policy](#)
- [Mental Health Patient Flow and Bed Capacity Framework](#)
- [Supporting transition from hospital \(7 day follow up\) Guideline](#)
- [Acute Response Mental Health Emergencies/Crisis Policy](#)

WA Department of Health Policy Hub

- [Clinical Handover Policy](#)
- [Patient Activity Data Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) for Mental Health Policy](#)
- [Triage to Discharge: MH Framework for SSCD](#)
- [Review of Death Policy](#)

Other

- [Inquest into the death of Paul Strange \(September 2019\) - Coroner's Court of Western Australia](#)
- [Guidelines for best practice psychosocial assessment of Aboriginal and Torres Strait Islander people presenting to hospital with self-harm and suicidal thoughts - . Menzies School of Research, Darwin](#)
- [Clinical care of people who may be suicidal: Policy Directive - NSW Health](#)
- Emergency Procedures – [RPH](#), [BHS](#), [TCU](#)

Clinical Care of People Who May Be Suicidal Policy

Version: 3.0

Endorsed by: **Clinical Practice Committee 05/09/2024**

Authorised by: **Deliver What Matters Most Committee 24/09/2024**

Date Compiled:

09/2018

Executive Sponsor

MH Division Medical Co-Director

Revision Due:

09/2027

Related forms

- *Patient Initiated Discharge Form*
- *Behavioural Observation Form*
- *What Matters Most Record*
- *Death in Hospital Form*

Links to resources

- [Assessing and Responding to the Suicidal Person: eLearning Modules – EMHS MyLearning](#)
- [Assessing and Responding to the Suicidal Person: Blended learning – EMHS MyLearning](#)
- [Take 5: Suicide Assessment](#)

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy Hub.

Related national standards

National Standards for Mental Health Services (2010): Standard 2 - Safety; Standard 4 – Diversity Response, Standard 6 - Consumers; Standard 7 – Carers, Standard 10 - Delivery of care

ACSQHC NSQHS Standards 2nd Edition (2021); Standard 2 - Partnering with Consumers; Standard 5 - Comprehensive Care; Standard 8 - Recognising and Responding to Acute Deterioration

References

1. Government of WA MHC. WA Suicide Prevention Framework 2021-2025. <https://www.mhc.wa.gov.au/media/1220/suicide-prevention-2020-strategy-final.pdf> [wa-suicideprevention-a4-mk40-web-version.pdf \(mhc.wa.gov.au\)](https://www.mhc.wa.gov.au/media/1220/suicide-prevention-2020-strategy-final.pdf) Accessed 17 Aug 2024
2. David A. Jobes [Commonsense Recommendations for Standard Care of Suicidal Risk | Journal of Health Service Psychology](#) . Journal of Health Service Psychology (2020) 46:155–163. <https://doi.org/10.1007/s42843-020-00020-3>. Accessed 17 Aug 2024
3. Teresa Sufrate-Sorzano, Iván Santolalla-Arnedo, María Elena Garrote-Cámara, Beatriz Angulo-Nalda, Ruth Coteló-Sáenz, Roland Pastells-Peiró, Filip Bellon, Joan Blanco-Blanco, Raúl Juárez-Vela, Fidel Molina-Luque. [Interventions of choice for the prevention and treatment of suicidal behaviours: An umbrella review - PMC \(nih.gov\)](#) Nurs Open. 2023 Aug; 10(8): 4959–4970. Published online 2023 May 22. doi: 10.1002/nop2.1820. PMCID: PMC10333855 PMID: 37218123. Accessed 27 Aug 2024

Appendix I: Examples of clinical interviewing questions for persons who may be suicidal

- Reassure and normalise the suicidal experience and expand the person's verbalisation about their suicidal thoughts and/or behaviours as *the* expert of their experiences, including but not limited to:
 - I know it may be difficult to talk about such personal issues, particularly with someone you just met. It takes a lot of personal courage to do so
 - Can you tell me about what your suicidal thinking is like for you? When does it typically come up for you?
 - Clarifying or explaining background of suicidal thoughts or behaviours/ self-harm
 - Do you have a suicide plan?
 - Do you have access to means required to enact a suicide plan?
 - Have you experienced any recent crises, losses, or stressors?
 - Do you feel hopelessness, extreme sadness, or despair?

Note: initial assessment questions must be clear and direct.

 - I see what you are experiencing, and I understand why you have these suicidal thoughts – what I feel is in the best interest is for us to find a way to effectively treat what makes you suicidal.
- Discuss with the person what would help them feel safer. Never endorse suicide
- Assess other potential risk factors such as
 - Deterioration in mental health – demonstrated or reported
 - Impulsivity
 - Changes in behaviour e.g. increased substance use
 - Changes in sleep patterns
 - Lack of social support or family support/social isolation – e.g. asking:
 - Do you have family with you?
 - Are there any of your circumstances now that make you feel unsafe or alone?
 - Do you have any concerns regarding your safety once you have been discharged?
- Be aware of language used, as some people view labels and assessments as problematic and reductionist – disliking certain terms, e.g. 'risk' and 'risk assessment'